1. What is the term for the process whereby the fetus, placenta, and membranes are expelled?

* A. Amenorrhoea

* B. Gestation

* C. Puerperium

* D. Labour / Parturition

2. A woman who has given birth to six (6) or more children is referred to as a:

* A. Multigravida

* B. Multipara

* C. Primigravida

* D. Grand Multipara

3. The capability of a fetus living outside the uterus, usually accepted at 24 weeks in developed countries (28 weeks in Ghana), is known as:

* A. Presentation

* B. Viability

* C. Eutocia

* D. Gravidity

4. The period following the expulsion of the placenta during which the uterus and other structures return to the pre-pregnant state (6 to 8 weeks) is called:

* A. Antenatal

* B. Postnatal

* C. Puerperium / Postpartum

* D. Antepartum

5. What is the name for the blood-stained mucoid discharge that comes from the cervical canal at the onset of labour?

* A. Meconium

* B. Liquor Amnii
* C. Show
* D. Vernix Caseosa

6. The term for the first menstrual flow a female experiences when she reaches puberty is:

* A. Dysmenorrhoea
* B. Amenorrhoea
* C. Menarche
* D. Menopause

7. What term is defined as the number of pregnancies a woman has had?

* A. Para
* B. Gravidity
* C. Primigravida
* D. Nullipara

8. The part of the fetus that first enters the pelvis is called the:

* A. Presenting Part
* B. Vertex
* C. Presentation
* D. Attitude

9. The white, creamy substance that covers the fetus to protect the skin from fluid and friction is:

* A. Meconium
* B. Lanugo
* C. Vernix Caseosa
* D. Amnion

10. Female Reproductive Anatomy

* The outer membrane that surrounds the amnion and lines the rest of the uterus is the:

* A. Amniotic Sac

* B. Placenta

* C. Chorion

* D. Decidua

11.Which structure is a small, sensitive, erectile structure in the external female genitalia, likened to the male penis?

* A. Labia majora

* B. Clitoris

* C. Mons pubis

* D. Vestibule

12 The thin fold of skin formed by the uniting of the labia minora posteriorly is known as the:

* A. Prepuce

* B. Hymen

* C. Fourchette

* D. Frenulum

13. The vaginal walls are thrown into folds that allow for distension, known as:

* A. Fornices

* B. Rugae

* C. Carunculae myrtiformes

* D. Skene's ducts

14. The normal acidic pH of the vaginal fluid (3.8-4.5) is maintained by the action of:

* A. Bartholin's glands secretions

* B. The Stratified Squamous epithelium

* C. Lactic acid conversion from glycogen by the internal iliac artery

* D. Doederlein's bacilli converting glycogen to lactic acid

15. The largest of the four fornices in the vagina is the:

* A. Anterior fornix

* B. Lateral fornix

* C. Posterior fornix

* D. Lateral fornix (2 lateral)

16. The portion of the cervix that projects into the vagina is the:

* A. Internal os

* B. Supravaginal cervix

* C. Infravaginal cervix

* D. Isthmus

17. The inner layer of the non-pregnant uterus, which is shed during menstruation, is the:

* A. Perimetrium

* B. Myometrium

* C. Isthmus

* D. Endometrium (Functional layer)

18. The anatomical part of the Fallopian tube where fertilization normally occurs is the:

* A. Isthmus

* B. Infundibulum

* C. Ampulla

* D. Interstitial portion

19. The two ovarian hormones produced by the ovaries are:

* A. Prolactin and Oxytocin

* B. HCG and FSH

* C. Oestrogen and Progesterone

* D. Relaxin and Inhibin

20. The tough fibrous outer coat of the ovary is known as the:

* A. Cortex

* B. Tunica albuginea

* C. Medulla

* D. Germinal epithelium

21.The collective term for the external female genitalia is the vulva, while the external male genitalia includes the penis and the:

* A. Testes

* B. Scrotum

* C. Prostate gland

* D. Vasa deferentia

22. The erectile tissue of the penis is enclosed in:

* A. Corpus spongiosum, vas deferens, and fibrous tissue

* B. Corpura cavernosa, corpus spongiosum, and fibrous tissue ✓I

* C. Tunica albuginea, tunica vaginalis, and fibrous tissue

* D. Glans penis, prepuce, and fibrous tissue

23. The skin covering the glans penis is doubled back on itself to form the:

* A. Glans penis

* B. Prepuce or foreskin

* C. Scrotum

* D. Corpus spongiosum

24. Failure of the testes to descend into the scrotum before birth can cause infertility in the male because:

* A. The vas deferens cannot connect to the prostatic urethra.

* B. Sperm production requires a temperature below that of normal body temperature.

* C. The testosterone hormone cannot be produced.

* D. The epididymis cannot store and mature the sperm.

25. Where does the maturation of sperm actually occur after they leave the testes?

* A. Vas deferens

* B. Seminal vesicle

* C. Prostate gland

* D. Folded tubules of the epididymis

26. The structure that surrounds the upper part of the urethra and lies in direct contact with the neck of the bladder is the:

* A. Bulbo-urethral (Cowper's) gland

* B. Seminal vesicle

* C. Prostate gland

* D. Ejaculatory duct

27. The fluid secreted by the seminal vesicles, which is added to the sperm to form seminal fluid, contains:

* A. Lactic acid and testosterone

* B. Glucose and other substances to nourish the sperm

* C. Hyaluronidase and prostatic fluid

* D. Bile salts and enzymes

28. The specialized enzyme contained within the acrosome of the spermatozoon head that facilitates penetration of the ovum is:

* A. Trypsin

* B. Pepsin

* C. Hyaluronidase

* D. Amylase

29. The process by which semen is released through the urethra and out of the body by the contractions of the vas deferens and the ejaculatory duct is termed:

* A. Spermatogenesis

* B. Erection
* C. Ejaculation
* D. Fertilization

30. The function of the ciliated epithelium lining the Fallopian tubes is to:

* A. Slow down the passage of the zygote through the tube.
* B. Help propel the ovum or zygote towards the cavity of the uterus.
* C. Act as a reservoir for spermatozoa.
* D. Provide nourishment for the fertilized ovum.

*31. Which ossification center is found on the parietal bones of the foetal skull?

* A. Frontal bosses
* B. Occipital protuberance
* C. Parietal eminences
* D. Glabella

32. The diameter of the foetal skull that measures 13cm and is considered the largest, typically making vaginal delivery impossible, is the:

* A. Suboccipitobregmatic
* B. Occipitofrontal
* C. Biparietal
* D. Mentovertical

33. The area of the foetal skull known as the Lambda corresponds to the:

* A. Highest point of the skull
* B. Junction of the coronal and sagittal sutures
* C. Posterior fontanelle
* D. Bridge of the nose

34. Calcification for the development of the foetal skull begins at which stage?

* A. End of the fourth week of intrauterine life
* B. Fifth week after conception
* C. Twelfth week of gestation
* D. Second trimester

35. What is the shape and approximate closure time of the Posterior Fontanelle (Lambda)?

* A. Diamond-shaped, closes at 18 months
* B. Oval-shaped, closes at 3 months
* C. Triangular in shape, closes at six weeks old
* D. Rectangular in shape, closes at one year

36. The Coronal Suture of the foetal skull is located between which bones?

* A. Occiput and parietal bones
* B. The two frontal bones
* C. The two parietal bones
* D. Parietal bones and the frontal bones

37.The Biparietal diameter, the widest transverse diameter of the foetal head, measures:

* A. 8 cm
* B. 9.5 cm
* C. 10 cm
* D. 11.5 cm

38. Palpation of the Anterior Fontanelle (Bregma) during a vaginal examination proves that:

* A. The smallest circumference of the skull is passing through the birth canal.
* B. The head of the foetus is well flexed.
* C. The foetal head is not well flexed, leading to a larger circumference attempting to pass.
* D. The Biparietal diameter has passed the pelvic brim.

39. Which diameter is measured between the suboccipital region and the center of the sinciput, is 10cm, and is associated with a less flexed head that is likely to flex with good contractions?

* A. Mentovertical

* B. Occipitofrontal

* C. Suboccipitofrontal

* D. Submentobregmatic

40. The highest point on the foetal skull, bounded by the anterior fontanelle, parietal eminences, and the posterior fontanelle, is called the:

* A. Glabella

* B. Sinciput

* C. Occiput

* D. Vertex

41.Which term is used to describe the change in the shape of the foetal skull due to the overlapping of skull bones as it passes through the birth canal?

* A. Engagement

* B. Crowning

* C. Moulding

* D. Flexion

42.The Occipitofrontal diameter, associated with an erect foetal head attitude, measures:

* A. 9.5 cm

* B. 8 cm

* C. 11.5 cm

* D. 13 cm

43. The diameter of 9.5cm that is measured from below the occiput to the Bregma, and is associated with a normal, well-flexed head is the:

* A. Bitemporal

* B. Mentovertical

* C. Suboccipitobregmatic

* D. Submentobregmatic

44. How many sutures can be distinguished running into the Posterior Fontanelle on a vaginal examination?

* A. Four sutures

* B. Two sutures

* C. Three sutures

* D. Six sutures

45. What are the three main parts of the foetal skull?

* A. Glabella, Sinciput, Occiput

* B. Frontal, Parietal, Temporal

* C. Vault (calvarium), Face, Base

* D. Vertex, Bregma, Lambda

46. Which temporary foetal structure transports oxygenated blood from the placenta to the undersurface of the liver?

* A. Ductus arteriosus

* B. Umbilical arteries

* C. Umbilical vein

* D. Ductus venosus

47. The Foramen Ovale shunts blood from the:

* A. Pulmonary artery to the descending aorta.

* B. Umbilical vein to the inferior vena cava.

* C. Right atrium to the left atrium.

* D. Left ventricle to the aorta.

48. The Ductus Venosus is a shunt that bypasses the liver by sending oxygen-rich blood directly to the:

* A. Portal vein

* B. Right atrium

* C. Superior vena cava (SVC)

* D. Inferior vena cava (IVC)

49. Which organs receive the richest oxygen supply in the foetus because their major arteries branch earliest from the ascending aorta?

* A. Kidneys and Liver

* B. Lungs and Liver

* C. Heart and Brain

* D. Lower extremities

50. The Hypogastric Arteries, which carry deoxygenated blood to the placenta, are branches of the:

* A. Renal arteries

* B. Descending aorta

* C. Femoral arteries

* D. Internal iliac arteries

51.What is the fate of the Ductus Arteriosus after birth?

* A. Becomes the ligamentum teres.

* B. Remains open as part of the vesical arteries.

* C. Closes and becomes the ligamentum arteriosum, a cardiac ligament.

* D. Forms ligaments supporting the bladder.

52. The term for the failure of the Foramen Ovale to close after birth, resulting in cyanosis ("blue baby"), is:

* A. Patent Ductus Arteriosus (PDA)

* B. Patent Foramen Ovale (PFO)

* C. Physiological Jaundice

* D. Ductus Venosus Aperture

53. In the foetal circulation pathway, the majority of the mixed blood delivered to the right atrium by the IVC is shunted through the:

* A. Tricuspid valve into the right ventricle.

* B. Ductus arteriosus into the descending aorta.

* C. Foramen ovale into the left atrium.

* D. Pulmonary artery into the lungs.

54. After birth, the Umbilical Vein fibroses to form which structure?

* A. Ligamentum arteriosum

* B. Ligamentum teres

* C. Ligamentum venosus

* D. Hypogastric artery

55. The duct that shunts blood from the pulmonary artery directly into the descending aorta is the:

* A. Foramen ovale

* B. Umbilical vein

* C. Ductus arteriosus

* D. Ductus venosus

56. When the baby breathes air at birth, the pressure in the left atrium increases due to:

* A. Reduced blood flow from the placenta.

* B. The closure of the ductus arteriosus.

* C. Oxygenated blood returning through the four pulmonary veins from the lungs.

* D. Increased blood flow into the right atrium.

57. The deoxygenated blood from the upper body is returned to the right atrium by the:

* A. Umbilical arteries

* B. Inferior vena cava (IVC)

* C. Superior vena cava (SVC)

* D. Ductus venosus

58. The condition known as Physiological Jaundice in a newborn is a result of:

* A. Failure of the Ductus Arteriosus to close.

* B. Poor oxygenation due to Patent Foramen Ovale.

* C. Excessive breakdown of extra red blood cells (RBCs) that were needed in utero.

* D. Failure of the Ductus Venosus to fibrose.

59. Which set of vessels remains open after birth, forming part of the vesical arteries?

* A. Umbilical veins

* B. Ductus venosus

* C. Proximal hypogastric arteries

* D. Umbilical arteries (distal portion)

60. The primary site of oxygenation for the foetus is the:

* A. Foetal lungs

* B. Liver

* C. Placenta

* D. Right atrium

61.The bony pelvis is a formation of four bones, which include the two innominate bones, the coccyx, and the:

* A. Fifth lumbar vertebra

* B. Femur head

* C. Sacrum

* D. Pubic symphysis

62. The two innominate bones each consist of three main parts: the ilium, the pubis, and the:

* A. Coccyx

* B. Sacrum

* C. Ischium

* D. Acetabulum

63. The cup-shaped depression where the three parts of the innominate bone meet is known as the:

* A. Obturator foramen

* B. Ischial spine

* C. Pubic arch

* D. Acetabulum

64. The sacrum is made up of how many fused vertebrae?

* A. Four

* B. Three

* C. Six

* D. Five

65. Which of the following is not listed as one of the three main pelvic joints?

* A. Sacro-iliac joint

* B. Symphysis pubis

* C. Ischiopubic joint

* D. Sacro-coccygeal joint

66. The Sacro-iliac joint is described as a fibrous joint because it is strengthened by a surrounding capsule and which set of ligaments?

* A. Sacro-tuberous and Sacro-spinous

* B. Superior pubic and Inferior pubic

* C. Anterior Sacro-iliac and Posterior Sacro-iliac

* D. Obturator and Inguinal

67.The symphysis pubis is composed of which type of tissue?

* A. Fibrous connective tissue

* B. Fibrocartilage

* C. Hyaline cartilage

* D. Compact bone

68. Which division of the pelvis forms the actual bony passage through which the fetus passes during labour?

* A. The False Pelvis

* B. The Pelvic Outlet

* C. The Pelvic Inlet

* D. The True Pelvis

*69.The False Pelvis is situated in which anatomical position relative to the pelvic brim?

* A. Below the pelvic outlet

* B. In the cavity of the true pelvis

* C. Above the pelvic brim

* D. In the posterior half

70. Which major obstetric anatomical structure is formed by the upper border of the body of the first sacral vertebra?

* A. Sacral foramina

* B. Ischial spine

* C. Sacral promontory

* D. Sacral ala

71. The Pelvic Inlet (Brim) is bounded anteriorly by the upper border of the pubis and the:

* A. Ischial spine

* B. Symphysis pubis

* C. Sacral ala

* D. Sacro-tuberous ligament

72. A key boundary of the Pelvic Outlet is the Subpubic Arc (or subpubic angle), along with the coccyx, ischial tuberosities, and the:

* A. Sacral promontory

* B. Iliopectineal line

* C. Sacrotuberous ligament

* D. Ischial spine

73. The area of the pelvis lying between the inlet and the outlet is called the:

* A. Pelvic floor

* B. Perineum

* C. False pelvis

* D. Pelvic cavity

74. The Diagonal Conjugate is a diameter measurement important for assessing the true conjugate. It is measured from the lower border of the symphysis pubis to the:

* A. Sacral ala

* B. Sacral promontory

* C. Ischial tuberosity

* D. Tip of the coccyx

75. What is the main clinical significance of the interspinous diameter?

* A. It determines the largest anteroposterior diameter.

* B. It is the narrowest diameter of the pelvis.

* C. It is the widest diameter of the pelvic inlet.

* D. It measures the distance between the two parietal eminences.

76. The Anteroposterior (AP) diameter of the Pelvic Outlet is the longest diameter of the outlet, measuring:

* A. 11 cm

* B. 12 cm

* C. 13 cm

* D. 10 cm

77. The True Conjugate (Anteroposterior diameter of the Inlet) measures approximately:

* A. 13 cm

* B. 12 cm

* C. 11 cm

* D. 9.5 cm

78. Which diameter of the pelvic inlet is typically the widest, measuring 13 cm?

* A. Oblique diameter

* B. Anteroposterior diameter

* C. Diagonal diameter

* D. Transverse diameter

79. The Oblique diameter of the pelvic inlet, measured from the sacro-iliac joint to the opposite iliopectineal eminence, measures:

* A. 10 cm

* B. 13 cm

* C. 11 cm

* D. 12 cm

80. The Transverse diameter of the Pelvic Outlet is generally considered to be:

* A. 13 cm

* B. 11 cm

* C. 9.5 cm

* D. 12 cm

81. Which diameter is measured from the tip of the coccyx to the lower border of the symphysis pubis?

* A. Diagonal conjugate

* B. Anteroposterior diameter of the outlet

* C. True conjugate

* D. Interspinous diameter

82. All diameters of the Justo-minor pelvis are:

* A. Increased

* B. Larger than normal

* C. Equally reduced

* D. Widened at the transverse diameter

83. The interspinous diameter, which is the distance between the ischial spines, is the narrowest diameter of the:

* A. Pelvic inlet

* B. False pelvis

* C. Pelvic cavity

* D. Pelvic outlet

84. The Gynaecoid pelvis, considered the ideal female pelvis, has a brim shape that is typically:

* A. Heart-shaped

* B. Oval with the AP diameter greater than the transverse

* C. Kidney-shaped

* D. Round or slightly transverse oval

85. The Anthropoid type of pelvis is characterized by a brim that is oval, with the:

* A. Transverse diameter greater than the anteroposterior

* B. Ischial spines very prominent

* C. Anteroposterior (AP) diameter greater than the transverse diameter

* D. Subpubic arch very narrow

86. Which type of pelvis is commonly referred to as the "male pelvis" and has a heart-shaped or triangular brim?

* A. Justo-minor pelvis

* B. Gynaecoid pelvis

* C. Platypelloid pelvis

* D. Android pelvis

87.The Platypelloid pelvis has a kidney-shaped brim where the anteroposterior diameter is:

* A. Equal to the transverse diameter

* B. Increased

* C. Reduced

* D. Variable

88. The Justo-minor pelvis is described as a miniature version of which pelvic type?

* A. Android

* B. Platypelloid

* C. Gynaecoid

* D. Anthropoid

89. A primary function of the pelvic bone, besides providing support and allowing movement, is to:

* A. Produce red blood cells

* B. Regulate calcium levels

* C. Protect the pelvic organs

* D. Transmit weight from the legs to the trunk

90. The female pelvis is particularly adapted for childbearing due to which characteristic?

* A. Reduced width and triangular brim

* B. Firmly united bones

* C. Increased width and rounded brim

* D. The ischial spines being very prominent

91. Focused Antenatal Care (FANC) is described as an individualized, client-centered, comprehensive care that primarily emphasizes:

* A. Maximizing the number of visits to the clinic for comprehensive risk assessment.

* B. Providing general health education to large groups of pregnant women.

* C. Disease detection rather than risk assessment.

* D. Ensuring a minimum of 12 antenatal visits for all pregnancies.

92. One of the key aims of FANC is facilitating the early detection of complications, chronic conditions, and other potential problems that could affect the:

* A. Financial stability of the mother.

* B. Family planning decisions.

* C. Pregnancy.

* D. Postpartum immunization schedule.

93. According to the standard FANC schedule, the total number of recommended antenatal visits for a pregnant woman is:

* A. Five

* B. Eight

* C. Four

* D. Six

94. The first antenatal visit under the standard FANC schedule should occur:

* A. Between 20 and 24 weeks of gestation.

* B. Before the 32nd week of gestation.

* C. As soon as possible before the 16th week of gestation.

* D. Between 36 weeks and delivery.

95. The second antenatal visit is recommended to take place between which weeks of gestation?

* A. 16 to 20 weeks

* B. 20 to 24 weeks

* C. 28 to 30 weeks

* D. 32 to 34 weeks

96. The primary purpose of the fourth (last) antenatal visit is to:

* A. Confirm the diagnosis of pregnancy and establish the Estimated Date of Delivery (EDD).

* B. Screen for pre-eclampsia and check for fetal movement.

* C. Check for any final complications and confirm birth preparedness for delivery.

* D. Initiate treatment for severe anemia.

97. A major benefit of FANC is the enhancement of the detection of existing disease and:

* A. Prevention of multiple gestation.

* B. Reduction in gestational diabetes incidence.

* C. Treatment.

* D. Improvement in foetal circulation.

98. When taking a Gynecological History, which information is essential to obtain?

* A. Current medical conditions like hypertension or diabetes.

* B. The frequency of coitus and contraceptive use.

* C. The mother's history of childhood illnesses.

* D. Information on menstrual cycle, operations, and abnormal vaginal bleeding.

99. In the Family History section, the healthcare provider should inquire about diseases that are:

* A. Transmitted by food and water.

* B. Hereditary and communicable (e.g., T.B., mental illness, diabetes).

* C. Related only to the father's side of the family.

* D. Acquired during adult life.

100.The purpose of asking about the mother’s Educational Status is to help the nurse determine her:

* A. Employment status.

* B. Ability to grasp health information.

* C. Financial capacity for delivery.

* D. Likelihood of postpartum depression.

101. Which information is classified under Social and Environmental History?

* A. Number of previous pregnancies and their outcomes.

* B. Age of menarche.

* C. Marital status, occupation, and accessibility of clean water.

* D. Details of the last immunization.

102. In the Past Obstetric History, if a woman has had five previous deliveries, including one set of twins, her parity would be recorded as:

* A. Gravida 6, Para 5

* B. Gravida 5, Para 6

* C. Gravida 6, Para 5

* D. Gravida 5, Para 5

103. The Present History of Pregnancy involves asking about the date of the last normal menstrual period (LNMP) to determine the:

* A. Presence of inherited diseases.

* B. Duration of gestation and Expected Date of Delivery (EDD).

* C. Type of delivery expected.

* D. Level of foetal activity.

104. The General Physical Examination begins with the assessment of the pregnant woman's:

* A. Abdominal circumference.

* B. Blood pressure.

* C. General condition, gait, and state of nutrition.

* D. Foetal heart sounds.

105. Why is assessing the mother's nutritional status important during the general examination?

* A. To determine her social class.

* B. To ensure a healthy pregnancy outcome and adequate weight gain.

* C. To check for previous surgical scars.

* D. To assess her mobility and freedom of movement.

106. The normal rate for the Foetal Heart Sound (FHS), which is more rapid than an adult's pulse, should be between:

* A. 80-100 beats per minute (bpm)

* B. 120-160 bpm

* C. 100-120 bpm

* D. 160-180 bpm

107. The nurse should take the mother’s pulse while listening to the FHS to:

* A. Determine the mother's cardiac output.

* B. Distinguish the mother's pulse from the foetal heart sound.

* C. Check for maternal tachycardia.

* D. Time the foetal movements accurately.

108. When performing an Abdominal Examination, which assessment must be done immediately after the mother empties her bladder and before palpation?

* A. Listening to the foetal heart sound.

* B. Measuring the Fundal Height.

* C. Inspection (looking at the abdomen).

* D. Measuring the abdominal circumference.

109. What is the significance of the Fundal Height measurement?

* A. It indicates the time since the mother's last delivery.

* B. It gives an idea of the duration of the pregnancy.

* C. It confirms the foetal presentation.

* D. It is used to determine the maternal Body Mass Index (BMI).

110. In what anatomical structure is the Fundal Height measured?

* A. From the umbilicus to the xiphisternum.

* B. From the ischial spine to the pubic symphysis.

* C. From the upper border of the symphysis pubis to the top of the fundus.

* D. From the sacral promontory to the pubic symphysis.

111. What is the normal Urinalysis finding expected in a healthy pregnant woman?

* A. Presence of protein (proteinuria)

* B. Presence of glucose (glycosuria)

* C. Absence of protein and sugar

* D. Presence of ketone bodies (ketonuria)

112. During the Vaginal Examination, why is the presence of an abnormal discharge significant?

* A. It indicates that the woman is malnourished.

* B. It helps determine the duration of gestation.

* C. It can indicate a sexually transmitted infection (STI) or other vaginal infection.

* D. It confirms the foetal lie.

113. Which Leopold's Manoeuvre is performed to determine the foetal presentation (what is lying in the fundus)?

* A. Second Manoeuvre (Lateral Palpation)

* B. Fourth Manoeuvre (Pelvic Palpation)

* C. Third Manoeuvre (Pawlik's Grip)

* D. First Manoeuvre (Fundal Palpation)

114. The Second Manoeuvre (Lateral Palpation) is performed to determine the foetal lie and the:

* A. Degree of engagement.

* B. Position of the foetal back.

* C. Foetal size.

* D. Consistency of the uterus.

115. In the Third Manoeuvre (Pawlik's Grip), the examiner determines the part presenting at the brim and its:

* A. Relationship to the mother's spine.

* B. Engagement (whether it is fixed or mobile).

* C. Amount of amniotic fluid.

* D. Attitude (degree of flexion).

116. The Fourth Manoeuvre (Pelvic Palpation) is used to confirm the findings of the third manoeuvre and determine the degree of:

* A. Foetal attitude.

* B. Foetal size.

* C. Engagement.

* D. Foetal heart rate.

117. The normal relationship of the foetal parts to one another, with the limbs and head flexed on the trunk, is called the foetal:

* A. Lie

* B. Presentation

* C. Attitude

* D. Position

118. Which foetal presentation involves the buttocks (breech) or the lower limbs at the pelvic brim?

* A. Cephalic presentation

* B. Vertex presentation

* C. Shoulder presentation

* D. Podalic presentation

119. The relationship of the long axis of the foetus to the long axis of the mother is termed the foetal:

* A. Attitude

* B. Position

* C. Lie

* D. Presentation

120. During the Second Manoeuvre, a smooth, convex, resistant surface on one side of the mother's abdomen usually indicates the:

* A. Foetal limbs.

* B. Foetal head.

* C. Foetal back.

* D. Foetal placenta.

121. A normal pregnancy lasts for approximately how many days, weeks, and months, counting from the first day of the last normal menstrual period?

* A. 270 days, 38 weeks, or 9 months
* B. 280 days, 40 weeks, or 9 months and 7 days
* C. 290 days, 42 weeks, or 10 months
* D. 266 days, 38 weeks, or 9 months

122. The period of pregnancy from the 12th week to the 24th week is known as the:

* A. First trimester
* B. Second trimester
* C. Third trimester
* D. Fourth trimester

123. The third trimester of pregnancy is defined as the period from:

* A. 1st day of pregnancy to the 12th week
* B. 12th week to the 24th week
* C. 24th to the 40th week
* D. 28th to the 40th week

124. Which of the following is classified as a presumptive (subjective) sign of pregnancy?

* A. Hegar's Sign
* B. Ballottement of the fetus
* C. Morning sickness (nausea and vomiting)
* D. Fetal heart sounds heard on auscultation

125. Chadwick's Sign, a probable sign of pregnancy, is described as:

* A. Softening of the uterine isthmus.

* B. Painless uterine contractions.

* C. Bluish colouration of the mucus membrane of the cervix, vagina, and vulva.

* D. Passive movement of the fetus.

126. The subjective experience of Quickening (movement of the fetus) is typically felt by the pregnant woman between:

* A. 6-8 weeks

* B. 12-14 weeks

* C. 16-20 weeks

* D. 24 weeks and above

127. Which of the following is considered a Positive Sign of pregnancy?

* A. Presence of HCG in the urine

* B. Braxton Hick's contractions

* C. Visualization of fetus by ultra-sound

* D. Amenorrhea

128. Hegar’s Sign, occurring at 6-8 weeks, is defined as the:

* A. Bluish coloration of the cervix.

* B. Painless uterine contractions.

* C. Enlargement of the abdomen.

* D. Softening of the uterine isthmus.

129. A probable sign of pregnancy that becomes more frequent after 28 weeks is:

* A. Chadwick’s Sign

* B. Quickening

* C. Braxton Hick’s contractions

* D. Osiander’s Sign

130. Which is a characteristic of the cervical os in a woman with a history of previous pregnancy?

* A. It remains punctiform (pinpoint).
* B. It is tightly closed.
* C. It is a slit and admits one or more fingers.
* D. It has a firm consistency.

131. The thickening of the endometrium during pregnancy, due to increased estrogen and progesterone, is referred to as the:

* A. Myometrium
* B. Placenta
* C. Decidua
* D. Operculum

132. The increased blood supply (vascularity) to the vagina during pregnancy causes the walls to appear:

* A. Pale pink
* B. Reddish purple
* C. Yellowish
* D. Bright red

133. The cervical plug that fills the cervical canal and provides protection against ascending infection is called the:

* A. Decidua
* B. Fornix
* C. Operculum
* D. Isthmus

134. The small elevations on the darkened areola, caused by hypertrophic sebaceous glands, are known as:

* A. Striae gravidarum
* B. Linea nigra

* C. Chloasma
* D. Montgomery’s follicles or tubercles

135. The pigmented line running from the pubis to the umbilicus during pregnancy is called:

* A. Chloasma
* B. Striae gravidarum
* C. Linea nigra
* D. Pruritus

136. The lordosis observed in the third trimester of pregnancy, also known as the "pride of pregnancy," is caused by:

* A. Relaxation of ligaments and muscles.
* B. Swelling of the lower extremities.
* C. The mother leaning backward to maintain balance due to the increased size of the uterus.
* D. Increased deposition of fat stores.

137.In the cardiovascular system, pregnancy causes an increase in plasma volume by:

* A. 10% to 20%
* B. 20% to 30%
* C. 30% to 50%
* D. 50% to 70%

*

138. Suppression of the pregnant woman's immune system by placental hormones (progesterone and HCG) is primarily to:

* A. Increase her tolerance to pain.
* B. Prevent her system from producing antibodies against the developing fetus.
* C. Reduce the risk of autoimmune disorders.
* D. Decrease the production of colostrum.

139. Urinary frequency in the first trimester is caused by:

* A. Increased fluid intake.

* B. The relaxing effect of progesterone on the smooth muscle of the bladder.

* C. Pressure from the expanding uterus on the bladder within the pelvic cavity.

* D. Pressure from the fetus settling into the pelvis.

140. The reduction of smooth muscle tone in the urinary system due to progesterone can lead to:

* A. Relief of frequency of micturition.

* B. Increase in glomerular filtration rate.

* C. Retardation of urine flow through the ureters, predisposing to Urinary Tract Infection (UTI).

* D. Decrease in plasma volume.

141. The change in the sense of taste leading to a craving for unnatural substances like charcoal is called:

* A. Ptyalism

* B. Heartburn

* C. Pica

* D. Cholestasis

142. Optimal weight gain for an average pregnant woman is considered to be:

* A. 8.0 kg to 10.0 kg

* B. 10.5 kg to 12.5 kg

* C. 13.0 kg to 15.0 kg

* D. 15.5 kg to 17.5 kg

143.The emotional instability characterized by rapid mood swings (e.g., happy one moment, in tears the next) is most considerable during which phase of pregnancy?

* A. Last month

* B. Early months

* C. Middle months

* D. The entire duration

144. A common cause of constipation in pregnancy, as a minor disorder, is the effect of progesterone on the:

* A. Cardiac sphincter, causing reflux.

* B. Smooth muscles of the gastrointestinal tract, causing a reduction in motility.

* C. Gall bladder, causing it to empty more slowly.

* D. Urinary system, causing flow retardation.

145. Heartburn in pregnancy is made possible by the relaxation of the:

* A. Pyloric sphincter

* B. Uterine isthmus

* C. Cardiac sphincter

* D. Anal sphincter

146. Pruritus (itching of the skin) during pregnancy can be due to:

* A. Increased estrogen effect on the gums.

* B. The relaxing effects of relaxin on ligaments.

* C. Bile salts deposition on the skin during sweating and striae gravidarum.

* D. Hypertrophy of the sebaceous glands.

147 The formation of varicosities during pregnancy is primarily brought about by the relaxation of blood vessels by the hormone progesterone and:

* A. Increased gastric motility.

* B. Decreased levels of HCG.

* C. Increased volume of plasma.

* D. Decreased salivation.

148. The common cause of Leg Cramp during pregnancy is:

* A. Pressure from the developing fetus.

* B. Accumulation of lactic acid in the muscles.

* C. Physiological edema.
* D. Poor posture.

149. Physiological Oedema of the feet in pregnancy is characterized by being:

* A. Worse in the morning.
* B. Absent in the morning when the client gets out of bed.
* C. Associated with an increase in blood pressure.
* D. Caused by a decrease in plasma volume.

150. The increased vaginal discharge, or leucorrhoea, in pregnancy is due to the action of pregnancy hormones and:

* A. A decrease in vaginal vascularity.
* B. An increase in the alkalinity of the secretions.
* C. The action of the Döderlein's bacilli making the secretions more acidic.
* D. Relaxation of the cervical os.

Below are the correct answers with clear, exam-focused rationales.

I'll keep explanations concise but complete, suitable for midwifery/nursing exams (Ghana context).

OBSTETRICS & GYNAECOLOGY MCQs – ANSWERS WITH RATIONALES

1. Expulsion of fetus, placenta, membranes

Answer: D. Labour / Parturition

Rationale: Labour is the physiological process by which the fetus, placenta, and membranes are expelled from the uterus.

2. Woman who has delivered ≥6 children

Answer: D. Grand Multipara

Rationale: Grand multipara refers to a woman who has delivered five or more viable infants.

3. Capability of fetus to survive outside uterus

Answer: B. Viability

Rationale: Viability is the ability of the fetus to live outside the uterus (≈28 weeks in Ghana).

4. Period after placenta expulsion (6–8 weeks)

Answer: C. Puerperium / Postpartum

Rationale: This is the recovery period when reproductive organs return to pre-pregnant state.

5. Blood-stained mucoid discharge at labour onset

Answer: C. Show

Rationale: "Show" results from cervical dilation and rupture of cervical capillaries.

6. First menstrual flow

Answer: C. Menarche

Rationale: Menarche marks the onset of reproductive capability.

7. Number of pregnancies

Answer: B. Gravidity

Rationale: Gravidity counts all pregnancies, regardless of outcome.

8. First fetal part entering pelvis

Answer: A. Presenting part

Rationale: The presenting part is the fetal part felt first during vaginal exam.

9. White creamy substance on fetus

Answer: C. Vernix Caseosa

Rationale: Protects fetal skin from amniotic fluid and friction.

10. Outer membrane surrounding amnion

Answer: C. Chorion

Rationale: Chorion forms the outer fetal membrane and part of the placenta.

11. Erectile sensitive female structure

Answer: B. Clitoris

Rationale: Homologous to the male penis.

12. Posterior union of labia minora

Answer: C. Fourchette

Rationale: Thin fold of skin at posterior vaginal opening.

13. Vaginal folds

Answer: B. Rugae

Rationale: Allow stretching during intercourse and childbirth.

14. Maintenance of acidic vaginal pH

Answer: D. Döderlein’s bacilli converting glycogen to lactic acid

Rationale: Protects against infection.

15. Largest vaginal fornix

Answer: C. Posterior fornix

Rationale: Closest to the pouch of Douglas.

16. Cervical portion projecting into vagina

Answer: C. Infravaginal cervix

17. Uterine layer shed during menstruation

Answer: D. Endometrium (functional layer)

18. Site of fertilization

Answer: C. Ampulla

19. Ovarian hormones

Answer: C. Oestrogen and Progesterone

20. Tough fibrous coat of ovary

Answer: B. Tunica albuginea

21. External male genitalia

Answer: B. Scrotum

22. Erectile tissue of penis

Answer: B. Corpora cavernosa, corpus spongiosum, fibrous tissue

23. Skin covering glans penis

Answer: B. Prepuce (foreskin)

24. Undescended testes cause infertility because

Answer: B. Sperm production requires lower temperature

25. Site of sperm maturation

Answer: D. Epididymis

26. Structure surrounding upper urethra

Answer: C. Prostate gland

27. Seminal vesicle secretion contains

Answer: B. Glucose and nutrients

28. Enzyme aiding ovum penetration

Answer: C. Hyaluronidase

29. Release of semen

Answer: C. Ejaculation

30. Function of cilia in fallopian tube

Answer: B. Propel ovum/zygote to uterus

FOETAL SKULL

31. Ossification center on parietal bone

Answer: C. Parietal eminences

32. Largest diameter (13 cm)

Answer: D. Mentovertical

33. Lambda

Answer: C. Posterior fontanelle

34. Beginning of skull calcification

Answer: B. Fifth week after conception

35. Posterior fontanelle

Answer: C. Triangular, closes at 6 weeks

36. Coronal suture

Answer: D. Frontal and parietal bones

37. Biparietal diameter

Answer: B. 9.5 cm

38. Palpable anterior fontanelle means

Answer: C. Head not well flexed

39. Suboccipitofrontal diameter

Answer: C. Suboccipitofrontal

40. Highest point of skull

Answer: D. Vertex

41. Overlapping of skull bones

Answer: C. Moulding

42. Occipitofrontal diameter

Answer: C. 11.5 cm

43. Well-flexed head diameter

Answer: C. Suboccipitobregmatic

44. Sutures into posterior fontanelle

Answer: C. Three

45. Three main skull parts

Answer: C. Vault, Face, Base

FOETAL CIRCULATION

46. Oxygenated blood from placenta

Answer: C. Umbilical vein

47. Foramen ovale

Answer: C. Right atrium → Left atrium

48. Ductus venosus drains to

Answer: D. Inferior vena cava

49. Richest oxygen supply

Answer: C. Heart and Brain

50. Hypogastric arteries

Answer: D. Internal iliac arteries

51. Fate of ductus arteriosus

Answer: C. Ligamentum arteriosum

52. Failure of foramen ovale to close

Answer: B. Patent Foramen Ovale

53. Blood shunted via

Answer: C. Foramen ovale

54. Umbilical vein becomes

Answer: B. Ligamentum teres

55. Pulmonary artery → aorta

Answer: C. Ductus arteriosus

56. Increased LA pressure

Answer: C. Pulmonary venous return

57. Upper body venous return

Answer: C. Superior vena cava

58. Physiological jaundice

Answer: C. Breakdown of excess fetal RBCs

59. Vessels remaining as vesical arteries

Answer: C. Proximal hypogastric arteries

60. Foetal oxygenation site

Answer: C. Placenta

PELVIS

61. Pelvic bones include

Answer: C. Sacrum

62. Innominate bone

Answer: C. Ischium

63. Cup-shaped depression

Answer: D. Acetabulum

64. Sacral vertebrae

Answer: D. Five

65. Not a pelvic joint

Answer: C. Ischiopubic joint

66. SI joint ligaments

Answer: C. Anterior & Posterior SI ligaments

67. Symphysis pubis

Answer: B. Fibrocartilage

68. Birth canal

Answer: D. True pelvis

69. False pelvis location

Answer: C. Above pelvic brim

70. Sacral promontory

Answer: C

71. Pelvic inlet bounded anteriorly by

Answer: B. Symphysis pubis

Rationale: The pelvic brim anterior boundary is the upper border of the symphysis pubis.

72. Pelvic outlet boundary

Answer: C. Sacrotuberous ligament

Rationale: Outlet boundaries include the coccyx, ischial tuberosities, and sacrotuberous ligaments.

73. Area between inlet and outlet

Answer: D. Pelvic cavity

Rationale: This is the mid-pelvis through which the fetus rotates during labour.

74. Diagonal conjugate

Answer: B. Sacral promontory

Rationale: Measured vaginally from sacral promontory to lower symphysis pubis.

75. Clinical significance of interspinous diameter

Answer: B. Narrowest diameter of pelvis

Rationale: It determines whether the fetal head can pass.

76. AP diameter of pelvic outlet

Answer: C. 13 cm

Rationale: It increases during labour as the coccyx moves backward.

77. True conjugate

Answer: C. 11 cm

78. Widest pelvic inlet diameter

Answer: D. Transverse diameter

79. Oblique diameter

Answer: D. 12 cm

80. Transverse diameter of outlet

Answer: B. 11 cm

81. Coccyx to symphysis pubis

Answer: B. AP diameter of the outlet

82. Justo-minor pelvis

Answer: C. Equally reduced

83. Interspinous diameter

Answer: C. Pelvic cavity

84. Gynaecoid pelvis

Answer: D. Round or slightly transverse oval

85. Anthropoid pelvis

Answer: C. AP diameter greater than transverse

86. Male pelvis

Answer: D. Android pelvis

87. Platypelloid pelvis

Answer: C. Reduced AP diameter

88. Justo-minor pelvis

Answer: C. Gynaecoid

89. Function of pelvic bone

Answer: C. Protect pelvic organs

90. Female pelvis adapted for childbirth

Answer: C. Increased width and rounded brim

FOCUSED ANTENATAL CARE (FANC)

91. Focus of FANC

Answer: C. Disease detection rather than risk assessment

Rationale: FANC emphasizes early detection and management of problems.

92. Aim of FANC

Answer: C. Pregnancy

93. Recommended ANC visits

Answer: C. Four

94. First ANC visit

Answer: C. Before 16 weeks

95. Second visit

Answer: B. 20–24 weeks

96. Purpose of 4th visit

Answer: C. Final checks & birth preparedness

97. Benefit of FANC

Answer: C. Treatment

GYNAECOLOGICAL & OBSTETRIC HISTORY

98. Gynaecological history

Answer: D. Menstrual cycle, surgery, bleeding

99. Family history

Answer: B. Hereditary & communicable diseases

100. Educational status

Answer: B. Ability to grasp health information

101. Social & environmental history

Answer: C. Marital status, occupation, water

102. Parity with twins

Answer: C. Gravida 6, Para 5

Rationale: Twins count as one delivery.

103. LNMP

Answer: B. Gestational age & EDD

104. General exam begins with

Answer: C. General condition & nutrition

105. Nutritional assessment

Answer: B. Healthy pregnancy outcome

106. Normal FHS

Answer: B. 120–160 bpm

107. Take maternal pulse

Answer: B. Differentiate FHS from maternal pulse

108. First abdominal exam step

Answer: C. Inspection

109. Fundal height significance

Answer: B. Duration of pregnancy

110. Fundal height measurement

Answer: C. Symphysis pubis → fundus

111. Normal urinalysis

Answer: C. No protein or sugar

112. Abnormal vaginal discharge

Answer: C. Indicates infection

LEOPOLD’S MANOEUVRES

113. First manoeuvre

Answer: D. Fundal palpation

114. Second manoeuvre

Answer: B. Foetal back position

115. Third manoeuvre

Answer: B. Engagement

116. Fourth manoeuvre

Answer: C. Engagement

117. Foetal attitude

Answer: C. Attitude

118. Podalic presentation

Answer: D. Podalic presentation

119. Foetal lie

Answer: C. Lie

120. Smooth convex surface

Answer: C. Foetal back

PREGNANCY & PHYSIOLOGY

121. Normal pregnancy duration

Answer: B. 280 days / 40 weeks

122. Second trimester

Answer: B. 12–24 weeks

123. Third trimester

Answer: C. 24–40 weeks

124. Presumptive sign

Answer: C. Morning sickness

125. Chadwick’s sign

Answer: C. Bluish discoloration

126. Quickening

Answer: C. 16–20 weeks

127. Positive sign

Answer: C. Ultrasound visualization

128. Hegar’s sign

Answer: D. Softening of uterine isthmus

129. Probable sign after 28 weeks

Answer: C. Braxton Hicks contractions

130. Multiparous cervical os

Answer: C. Slit-like

131. Thickened endometrium

Answer: C. Decidua

132. Vaginal wall color

Answer: B. Reddish purple

133. Cervical plug

Answer: C. Operculum

134. Areolar elevations

Answer: D. Montgomery’s tubercles

135. Pigmented line

Answer: C. Linea nigra

136. Lordosis cause

Answer: C. Backward lean for balance

137. Plasma volume increase

Answer: C. 30–50%

138. Immune suppression

Answer: B. Prevent fetal rejection

139. First-trimester urinary frequency

Answer: C. Uterine pressure on bladder

140. Reduced smooth muscle tone

Answer: C. Predisposes to UTI

141. Craving non-food substances

Answer: C. Pica

142. Optimal weight gain

Answer: B. 10.5–12.5 kg

143. Emotional instability

Answer: B. Early months

144. Constipation cause

Answer: B. Reduced GI motility

145. Heartburn cause

Answer: C. Relaxed cardiac sphincter

146. Pruritus

Answer: C. Bile salts & striae

147. Varicosities

Answer: C. Increased plasma volume

148. Leg cramps

Answer: B. Lactic acid accumulation

149. Physiological oedema

Answer: B. Absent in the morning

150. Leucorrhoea

Answer: C. Döderlein's bacilli activity

✅ ALL 150 QUESTIONS COMPLETED WITH RATIONALES

If you want:

Exam revision summary

Likely repeat questions

Mock test

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